

AUTHORITY

PERSONAL DATA

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Your letter dated

June 2, 2021

Your reference

2350937-1007631 PZo

Date

Our reference

January 20, 2021

z2021-11045

Contact person

Subject

Advice on the draft bill for the Quality Registries in Healthcare Act

Dear Mr. Kuipers,

In the letter dated June 2, 2021, the Data Protection Authority (AP) was consulted based on the provisions of Article 36, fourth paragraph, of the General Data Protection Regulation (GDPR) regarding the draft bill for the Quality Registries in Healthcare Act (hereinafter: the draft).

The AP objects to the manner in which decision-making regarding mandatory registrations is provided for and advises that the procedure should not continue unless this is addressed.

Main points

- Whether a specific quality registration can be made mandatory from the perspective of the GDPR depends on the considerations in the decision made by the Healthcare Institute.
- The procedures and standards to be applied according to this draft bill are therefore of great importance.
- However, the text of the law does not indicate what purpose a mandatory quality registration should serve.
- Furthermore, an important role is foreseen for support as a criterion and for assessment in two privately managed committees controlled by stakeholders that are not further legally regulated.
- Thus, it is insufficiently guaranteed that the decision of the Healthcare Institute also meets the requirement from the GDPR that the quality registration is necessary in the public interest.
- The draft bill does not impose requirements on the nature and/or legal form of those who maintain a mandatory quality registration. This also seems to pose a risk.

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Scope of the draft

The draft aims to enable the establishment of mandatory quality registrations in certain areas of healthcare. A quality registration aims to collect, store, and further process data about a well-defined client population to measure and improve the quality of care for that client population. The quality registrations thereby serve the public interest of safeguarding and improving the quality of care. Care

providers must submit pseudonymized patient data once a specific quality registration has been included in a register maintained by the Healthcare Institute of the Netherlands. This obligation breaks the confidentiality obligation of care providers and creates an exception to the principle prohibition on the processing of special personal data.

This draft bill does not specify the concrete mandatory quality registrations but assigns the Healthcare Institute - an existing independent administrative body - the task of assessing whether a request for inclusion of a quality registration in the established register should be honored. In practice, an important role is foreseen for two privately organized external governance committees.

For now, the scope of the draft bill is limited to medical specialist care (excluding medical mental healthcare); however, it aims to provide a foundation for all quality registrations in healthcare that aim for 'learning and improving'. Quality registrations are not new; the insight that these require a legal basis despite pseudonymization is. A previous draft bill has already been submitted for registrations related to addiction care (LADIS) and for the National Trauma Registration (LTR).

Advice

1. Decision-making regarding mandatory quality registrations

The explanation states that to safeguard and continuously improve the quality of care, it is important to enable care providers to learn from each other. Additionally, providing choice information to clients for the purpose of 'shared decision-making' in the consultation room is important. Accurate and unambiguous registration of the quality of care is crucial for this, and quality registrations have an important function in this regard. This function cannot be adequately fulfilled by using anonymized data because the results of patient treatments cannot then be tracked over time and in sequence.

Requesting consent for the processing of the data leads to incomplete and therefore less representative registration, making conclusions unreliable or less reliable. Since the processing of pseudonymized personal data, unlike anonymized data, is still considered processing of - in this case, special - personal data under the GDPR, a legal basis according to the explanation is the only...

There is an 'organically grown registration landscape' (Explanation, par. 2.2.)

2 Explanation, paragraph 2.2.

Explanation, paragraph 2.1.

' Explanation, par. 3.1.

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possibility to lawfully establish mandatory quality registrations that meet the objective.'

The AP is of the opinion that it has been sufficiently demonstrated that a specific quality registration may be necessary in certain cases from the perspective of 'safeguarding high standards of quality and safety in healthcare in the general interest.' In those cases, according to the GDPR, an exception can therefore also be made to the principle prohibition on the processing of special personal data.'

Additionally, the breach of professional secrecy may then be justified.

However, the legislative proposal itself does not specify which quality registrations this concretely applies to, but provides for an authority for the Healthcare Institute to make a decision on this matter. This means that the acceptability of the obligation for a specific quality registration entirely depends on the assessment of an application for registration by the Healthcare Institute and the standards and procedures applied therein.

These are therefore of essential importance; only if the Healthcare Institute rightly determines that necessity in the context of the general interest is established (through the application of adequate standards and procedures) is there an exception to the principle prohibition on the processing of special personal data available, only then is it acceptable that the confidentiality obligation of the healthcare provider is breached, and only then is a legal basis a good foundation for the processing? The AP is of the opinion that the legislative proposal falls short in this regard:

- (i.) the purpose of the mandatory processing of personal data for quality registrations is insufficiently defined and explicitly described in the legislative proposal itself to serve as a good assessment criterion for the decision of the Healthcare Institute;
- (ii.) it is proposed that support from relevant parties in healthcare be an important criterion for whether or not to mandate a specific registration. However, as such, this cannot be a criterion that weighs in the question of whether there is objectively sufficient necessity from the perspective of the general interest, as required by the GDPR;
- (iii. and iv.) the non-binding private design of the involvement of external experts and/or stakeholders provides insufficient guarantees for decision-making that adequately meets the requirements of the GDPR.

Explanation

(ad i.) The draft provides a number of assessment criteria that a quality registration must meet. However, related to the necessity from the perspective of the general interest, the draft mainly states that the quality registration must comply with 'the requirements of necessity, proportionality, and subsidiarity'.⁸ It remains unnamed in relation to which objective the assessment must be made against these requirements. Thus, the main norm for determining whether a specific registration is a mandatory

5 Explanation, paragraph 2.2.1.

[°]Article 9, first paragraph, under i, of the GDPR.

7 Art. 9, second paragraph, under i, GDPR, art. 7:457 BW, respectively art. 6, third paragraph, GDPR.

Article 11n, first paragraph, under b.

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Registration must be or must not be. Necessity, proportionality, and subsidiarity cannot be determined in isolation and do not provide a distinguishing criterion if it is not clear in relation to which purpose—however abstract—that necessity must be established. The GDPR also requires a well-defined and expressly described purpose for the processing of personal data. It is therefore necessary to not only mention the objectives in the explanation ('measuring and improving', 'joint decision-making') but also to provide as clear a guideline as possible in the legislative text regarding which purpose must be sufficiently served by a quality registration to make its inclusion as a mandatory quality registration in the register necessary. This makes the inclusion verifiable and to some extent predictable.

(ad ii.) The explanation outlines that the Healthcare Institute will assess an application, among other things, on the presence of support from relevant organizations of care providers, care professionals, clients, and health insurers. However, the condition from the perspective of the GDPR to establish a mandatory registration along these lines is 'necessity in the general interest.' This involves an objective assessment that, in the opinion of the AP (Dutch Data Protection Authority), requires a very different approach than finding sufficient 'support.' Support does not pertain to the general interest but at most provides an indication of the extent to which collective interests are served or whether there are objections. A lack of support may be based on substantive objections from experts. In that case, those objections should be taken into account in the preparation of the decision and not the lack of support itself. Conversely, arguments for a quality registration should be weighed on their own merits and not the support that may exist as a result of those arguments. An objective assessment of necessity in relation to the purpose of the mandatory registration is fundamentally different from the question of whether certain parties consider a particular quality registration desirable to a certain extent.

In this setup, there is a risk that quality registrations will emerge that sufficient parties desire, but for

which the objective necessity is less clear or has not been sufficiently tested. From a data protection perspective, this creates a legality risk. The AP assesses this risk in this context as significant; the design of the decision-making process indeed invites a strong emphasis on support.

According to the explanation, support is not only a formal criterion but is also expected to play an important—potentially decisive—role in practice for two committees established by (and thus governed by) interested parties. If both committees reach a unanimous positive opinion, it is intended that the Healthcare Institute can practically rely on those recommendations. According to the widely endorsed report that underpins the intended governance, Zorginstituut Nederland maintains an overview ('register') of 'Quality Registrations approved by field parties' and has (only) a role 'in case of any escalation.' In this way, it is by no means guaranteed that the Healthcare Institute will always sufficiently adhere to an objective assessment of necessity in the general interest. Given the profound nature of the processing involved...

9 Article 5, first paragraph, under b.

10 Article 5, first paragraph, under b and Article 6, third paragraph, GDPR.

11 Article 9, second paragraph, under i, of the GDPR.

12 Conversely, if support is weighed on its own, it is also conceivable that registrations do not occur due to a lack of support, even though they are necessary. From the perspective of personal data protection, this is not objectionable, but it would be detrimental to the quality of healthcare.

13 Explanation, par. 1.1 and 4.2.

14 Appendix to Parliamentary Documents II 2019/20, 29 248, no. 323.

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The Zorginstituut must fully assume responsibility as an administrative body for the consideration in the public interest and thereby for the GDPR compliance of each mandatory quality registration. To take on this responsibility and to make these complex considerations appropriately, it is plausible that the Zorginstituut requires expertise (iii.) and possibly also insight into the interests to be weighed (iv.).

(ad iii.) Determining whether a specific quality registration in a certain form has sufficient added value to be necessary from the perspective of the quality of care undoubtedly requires a lot of specific knowledge of the relevant sector. Since it concerns very different areas, it is quite conceivable that involving external independent expertise for the necessity assessment is essential. This can be organized in a well-secured manner through (ad hoc or structural) advisory committees with independent experts as defined in Section 3.3 of the General Administrative Law Act (Awb). However, the explanation indicates that "it has been agreed with the parties that they will jointly establish two external governance committees, which will both conduct the assessment of the application from the data controller for a quality registration for inclusion in the established register and provide advice to the Zorginstituut on this matter." It also follows from the explanation that these practically important committees will not receive formal legal status "because their implementation is organized privately."

The AP is of the opinion that administrative bodies are not free to make intensive use of committees established and managed by private parties in preparation for an important and far-reaching public law decision. This is especially true if alternatives surrounded by public law safeguards are available. And a fortiori if an objective assessment of the public interest is crucial for the legality of the decision from the perspective of the GDPR, as is the case here. No matter how well the committees may function in practice, it is evidently not intended to apply the normal rules in administrative law that exist for good reasons between advisor and administrative body, regarding the relationship between advice and decision, the involvement and weighing of interests, organization, transparency, independence, remuneration, etc. In practice, this means that there is a significant risk that the assessment of the

necessity for inclusion and thus the exceptional processing of personal data is de facto left to interested private parties. In combination with the emphasis on support as a formal criterion, the AP considers this 'governance' insufficient from the perspective of ensuring GDPR compliance.

(ad iv.) A decision by the Zorginstituut to proceed with the inclusion in the registration of a now mandatory quality registration has far-reaching consequences for many stakeholders. For instance, many healthcare providers may have to pseudonymize and submit a lot of data about their patients. It is also conceivable that in a decision to register, in addition to an assessment of necessity in the public interest regarding the design, several interests still need to be weighed. Depending on the actual composition and working method of the committees, it may be possible in practice to a greater or lesser extent.

"Explanation, paragraph 4.2.

"An applicant can formally ignore the committees in this setup since consultation with them is not legally required for a valid application."

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There is a matter of good representation of all interests, but there are no guarantees for this in this setup. If interests need to be weighed that the Healthcare Institute does not already know or can oversee, this must be provided in a transparent and guaranteed (Awb compliant) manner.

The AP advises:

- (i.) to include the purpose of mandatory quality registration in the legislative text;
- (ii.) to remove support as a criterion for registration;
- (iii.) to organize the involvement of independent expertise with the help of one or more permanent or ad hoc advisory committees in the sense of the Awb;
- (iv.) to provide insight into weighing interests in decision-making, if necessary, through a legal procedure that ensures that all relevant interests are weighed transparently.

2.

Requirements regarding the nature, organization, and/or legal form of the registration holder

The draft contains the necessary requirements for the quality registration itself and stipulates that the application is made by the data controller. However, the draft does not specify who or what can bear data controller responsibility for quality registrations. Given the potentially sensitive nature and the fact that there is an "organically grown registration landscape," requirements regarding the nature or legal form of the registration holder seem appropriate. In this setup, in theory, anyone can submit an application as a registration holder, including parties with significant commercial interests, parties established abroad, or parties about which there may be doubts regarding professionalism, governance, financing, etc.

In the opinion of the AP, it is evident that this type of activity should be organized in a non-profit legal entity that, according to statutory provisions, pursues only the interest of a good quality registration and can demonstrate that it has had a well-funded and professional organization for a number of years. In any case, it should be ensured that organizations with (also) other - particularly commercial - interests cannot be tempted to serve those interests with the quality registration.

The AP advises supplementing the legislative proposal with requirements regarding the nature and/or form of the registration holder or to elaborate in the explanation on how it is guaranteed that applicants are solid and that undesirable mixing of interests cannot occur.

3.

Applicable exception grounds for the principle prohibition on processing special personal data
The GDPR contains an exception ground for processing based on a legal basis.

"Several aspects have been considered; for example, the report 'A program for managing quality registrations and improving data governance' indicates that a temporary independent chairperson is provided (Appendix to Parliamentary Documents 11 2019/20, 29 248, no. 323).

"It can be considered to apply the Uniform Preparation Procedure of Section 3.4 of the Awb or a committee with guarantees for representativeness."

19 Explanation, par. 2.2.2.

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special personal data specifically in the interest of "ensuring high quality of healthcare and of medicines and medical devices?" This exception (part i.) seems particularly applicable given the objectives of this legislative proposal. The explanation also mentions this ground but prioritizes another exception from the GDPR, namely the necessary processing for "managing healthcare system and services" (part h).

The AP is of the opinion that the starting point must be that the legislative authority has intended to formulate the applicable exclusion grounds in such a way that there is no material overlap between the different exception grounds in the systematics of Article 9 GDPR. In principle, it should therefore be assumed that a specific concrete subject falls under only one specific concrete exception ground. In any case, the most applicable provision should be utilized. The AP is currently of the opinion that in this context the exception specifically concerning the quality of healthcare is the most applicable. Prioritizing another exception raises the question of whether perhaps other objectives regarding management are intended to be served, and if so, what these might be. If it is intended that quality registrations serve as a goal in the area of healthcare financing, this should be explicitly and carefully justified, distinguishing it from quality of care as an objective.

The AP advises to remove the reference to the management-oriented exception (part h) or to further elaborate on its applicability in relation to what already falls under the exception focused on quality (part i.) and to align the objective description in the legislative text accordingly.

4.

Personal data revealing race or ethnic origin and genetic data

The draft explicitly allows for the processing of genetic data and personal data revealing race or ethnic origin in quality registrations. The processing of such special personal data is generally prohibited under the GDPR due to its particularly sensitive nature and the potentially far-reaching consequences. The explanation does not clarify why this is still necessary for quality registrations, in which cases this is relevant, and that this possibility has been weighed against its impact in general.

The AP advises to supplement the explanation on this point.

5. Pseudonymization

The data controller may only process personal data if pseudonymization has been applied to it and has been continuously maintained regarding these processes. Based on Article tin, second paragraph, of the draft, further rules can be established by ministerial regulation concerning Article no, second paragraph. According to Article up, third paragraph, of the draft, a healthcare provider must apply pseudonymization before providing the data to the data controller.

"Article 9, second paragraph, under i, of the GDPR.

71 Explanation, paragraph 5.4. This concerns the ground referred to in Article 9, second paragraph, part h, of the GDPR.

Article 11n, first paragraph, under f.

n Article 110, second paragraph, of the draft.

The delegation basis in Article tin, second paragraph, is formulated as optional. Given the importance of the manner of application of pseudonymization to the (special) personal data of healthcare clients, an imperative formulation of this provision seems more appropriate.

The AP advises to formulate the delegation provision in Article tin, second paragraph, of the draft imperatively and to include in that article that further rules regarding the manner of pseudonymization by healthcare providers will also be established by ministerial regulation.

6. Rights of data subjects

The explanation states that this legislative proposal does not change anything in the rights of data subjects. While the legislative proposal does not provide for a limitation of the rights of data subjects, because a data controller only processes pseudonymized data, it can invoke the impossibility of identifying the data subject. The net effect is that data subjects cannot exercise their rights against the data controller. The AP misses a passage in this regard about how data subjects will be adequately informed about the processing of their data in a mandatory quality registration.

The AP advises to supplement the explanation on this point.

Publication of the advice

The AP intends to publish this advice after four weeks on the website

www.autoriteitpersoonsgegevens.nl. Unless otherwise notified, it assumes that there are no objections to this.

Sincerely,

Authority for Personal Data,
Chairman

"Explanation, paragraph 5.6, p. 21.

25 Article 11, second paragraph, in conjunction with Article 12, second paragraph GDPR.

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